

CD4	Use CD4 to guide co-trimoxazole preventive therapy (CPT), see table 5113.
Viral load	- If VL < 50, continue ART. - If VL ≥ 50, assess adherence to ART 5173 and discuss with experienced TB doctor or specialist.

Health for All

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Advise the patient with RR-TB

- Provide RR-TB counselling and arrange community health worker home visit. Refer to support group if available.
- Explain that duration of treatment will depend on previous treatment, site of disease and extent of drug resistance. Duration may need to be extended depending on response to treatment.
- Educate on the importance of adherence 5173 and dangers of further resistance. Educate about treatment side effects 5105, and advise to return promptly should they occur.
- Educate about infection control: cough hygiene, adequate ventilation/open windows, avoid close contact with children/those with HIV. Give surgical mask for use in poorly ventilated areas. Advise to avoid sharing a bedroom if possible.
- Advise that TB contacts¹ need to visit the clinic for TB screening/prevention.
- If pulmonary TB, advise to return to work only when culture conversion² occurs.
- Alert to the risks of smoking 5141 and alcohol/drugs and support patient to change 5177. If patient chooses to continue, advise safe alcohol use 5142 and to continue taking TB medication daily.

Treat the patient with RR-TB

- Give **pyridoxine** 50mg daily until TB treatment completed.
- Ensure COVID-19 vaccination is up to date.

If not on RR-TB treatment:

- Start treatment using steps 1-3 5102.
 - **Shorter** regimen is 9-11 months treatment (4-6 months intensive and 5 months continuation phase).
 - **Longer** regimen is 18-20 months treatment (6-8 months intensive and 12 months continuation phase).
 - If unsure of initial regimen choice, discuss with PCAC/NCAC.

If on RR-TB treatment:

- Check outstanding LPA and DST results³ and adjust regimen using step 2 5102.
- If patient has gained weight, check if medication doses need adjusting 5104.
- **Decide when to change intensive phase to continuation phase:**
 - If on shorter regimen: decide at end of month 4 5103.
 - If on longer regimen: decide at end of month 6 5103.

Treat the patient with RR-TB and HIV

Avoid EFV while on bedaquiline and AZT while on linezolid. Choose ART regimens according to the following scenarios:

Already on ART

If already on ART when RR-TB treatment started, check if ART regimen needs changing according to latest viral load results 5112.

Starting ART

- If starting ART after RR-TB treatment has been started, decide when to start ART 5115.
- Then choose most appropriate ART regimen: ideally choose **TLD** (TDF + 3TC + DTG). If eGFR < 50, choose instead **ABC** + 3TC + DTG.

ABC – abacavir; TDF – tenofovir; 3TC – lamivudine; TLD – tenofovir + lamivudine + dolutegravir

Re-starting ART after ART interruption

Provide adherence support 5173. Choose most appropriate ART regimen to re-start 5114.

Review the patient with RR-TB

- Assess patient at diagnosis, 2 weeks, 4 weeks and then monthly. Review sooner if not improving or any problems.
- Once RR-TB treatment complete, follow up 6 monthly (or earlier if any symptoms recur) for 2 years: at each visit check symptoms, do chest x-ray and send sputum for TB microscopy and culture.

Decide when to stop RR-TB treatment

- If on shorter regimen: stop treatment 5 months after changing to continuation phase if patient well and cultures remain negative. If unwell or cultures become positive, present to NCAC.
- If on longer regimen: stop treatment 12 months after changing to continuation phase if patient well and cultures remain negative. If unwell or cultures become positive, present to NCAC.
- Record treatment outcome 5103.

¹ A TB contact refers to a patient who shared an enclosed space (at work, socially, in a hostel, or in a household setting), for ≥ 1 night or for frequent/extended daytime periods, with an adult/adolescent with pulmonary TB ("index patient"), during the 3-month period before the index patient started their TB treatment. ² Culture conversion: 2 consecutive negative culture results one month apart. ³ If sample contaminated/inadequate/leaked or LPA results inconclusive, send another sample to laboratory.